CommonSpirit

February 20, 2026

Thomas Keane, MD, MBA
Assistant Secretary
Office of the Deputy Secretary and Assistant Secretary for Technology Policy
Department of Health and Human Services.
Mary E. Switzer Building; Mail Stop: 7033A
330 C Street S.W.
Washington, DC 20201

Re: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care (RIN 0955-AA13)

Dear Dr. Keane,

CommonSpirit Health appreciates the opportunity to provide comments to the Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology (ASTP/ONC) on its request for information (RFI) related to the use of artificial intelligence (AI) in clinical care. As a nationwide, faith-based, safety-net provider, we are committed to providing high quality care for the patients in our communities with a special focus on those who are most vulnerable or underserved. We believe that AI has a role to play in our care delivery, but it must be approached cautiously and thoughtfully. As one of the nation's largest health systems, CommonSpirit has specific experience in buying, using and evaluating the use of AI tools in clinical care. While some AI tools have been in place for a long time, those interacting with patients and clinical care are relatively nascent. We are pleased that ASTP/ONC is taking this opportunity to gather input from a diverse group of stakeholders at the early stages of technology development to ensure that any new tools used for patient care are safe and effective.

Our most fundamental recommendation to ASTP/ONC is to create and convene a technical expertise panel to develop specific transparency, patient safety, product liability, ethics, and other standards. This panel should include health systems, clinicians, vendors, technology experts, advocacy organizations, medical ethicists, and regulators. The concepts involved in clinical care AI are too specific, too important, and too complex to be regulated outside of a technical expertise panel. As a leader in the field, we offer CommonSpirit experts to participate in such a panel and aid ASTP/ONC in future rulemaking.

Secondary to this overarching recommendation of a technical expertise panel, we offer the following guidelines and protocols we believe ASTP/ONC must follow in any future rulemaking.

Require Robust Transparency

We strongly recommend that ASTP/ONC implement comprehensive transparency requirements for AI tools used in clinical decision-making. These rules must encompass a complete reporting of the algorithms and calculations themselves, the specific data sets used for their training (especially any use of synthetic data), an ongoing monitoring of outcomes, and a built-in evaluation process for users and patients to provide meaningful feedback to designers on an AI model's efficacy and accuracy. Furthermore, this transparency must address known AI issues, such as "hallucinations," by providing clear definitions and documentation of testing for these anomalies. The regulations should include definitions and best practices to ensure data cleanliness. Finally, as mentioned above, ASTP/ONC should work with a technical expertise panel to ensure that AI tools used for clinical decision-making are not biased by their data sets or underlying assumptions. We strongly recommend that regulatory bodies act on the critical importance of algorithmic and data transparency in clinical health AI tools and commit to a clear and robust stance on its enforcement.

(We note that the recent proposal in the HTI-5 update would roll back algorithmic transparency requirements for certified health IT. We seek clarity on whether there are plans to replace or update this requirement to ensure that the core mechanisms of these AI tools remain understandable and auditable by the clinical community and regulators.)

To ensure clinical rigor, patient safety, and data privacy, every new AI tool introduced into a clinical setting should be treated with a similar scrutiny and methodological intensity applied to a medical device undergoing a clinical trial. This requires full disclosure of the training and validation data sets, rigorous testing for validity, mandatory assessment of outcomes in a real-world setting, and ongoing monitoring of oversight (similar to a data safety monitoring board in clinical research). Only through this level of thorough vetting and continuous monitoring can the healthcare industry assure that AI tools enhance, rather than compromise, the quality and efficacy of patient care.

In essence, <u>mandatory and detailed transparency can not be an optional feature</u>—it is a critical prerequisite for the safe, ethical, and effective deployment of AI tools in medicine. Without robust transparency requirements into all non-proprietary elements of a clinically deployed AI tool, health care providers are fundamentally unable to meet their obligations concerning patient consent and notice, nor can providers assure patients of the safety and reliability of the technology being used in their treatment.

Support Patient Privacy

As AI tools become more common in clinical care, providers must help patients feel comfortable with these tools by ensuring that their data remains private. As a health system, one of our primary concerns is understanding precisely how AI vendors handle the sensitive protected health information (PHI) entrusted to them so we can monitor compliance with HIPAA and other privacy rules. **We strongly recommend that any data shared via an AI tool be completely de-identified before use in training or refining AI models, and that all vendors have well-defined, enforceable mechanisms for data purging in place once the data has served its agreed-upon purpose.** Without these foundational requirements, healthcare organizations risk severe erosion of patient confidence and potential data misuse, which could compromise our core mission of delivering high quality care to all.

Additionally, the expansive and quickly evolving digital health industry poses a significant challenge to the long-established guardrails of regulations like HIPAA. As healthcare moves beyond traditional physical settings and into complex digital ecosystems, new tools are constantly testing the boundaries of protected health information. **We urge regulators to ensure that technological advancement does not outpace ethical responsibility.** For example, current privacy laws primarily place responsibility on the covered entity (i.e. healthcare organization) for maintaining patient privacy. While this responsibility should not be lessened, an AI vendor that is not a covered entity currently has little to no legal responsibility to maintain patient privacy. **Extending the same privacy obligations and penalties to AI vendors would help ensure that patient data is used appropriately**. We believe it is imperative that healthcare leaders and regulators proactively work to ensure that the innovations brought by AI and digital health do not inadvertently create loopholes that weaken patient privacy and security.

Create Federal Standards

To meet the aforementioned goals, we recommend that regulators create a clear, standardized, and responsible regulatory framework that is intended to protect patients and providers alike. **We believe that regulators can foster safe and effective adoption of AI tools in clinical care by creating standards in three key areas: ensuring product efficacy, establishing vendor accountability, and standardizing data practices across the industry.**

First and foremost, health care patients and providers must be protected from ineffective or harmful “snake oil” AI solutions. Vendors should be required to provide evidence-based data proving the clinical effectiveness and usefulness of their products before they are adopted by healthcare providers. Without this proof of efficacy, providers risk wasting resources on unvalidated tools, and—more critically—introducing potential harm or bias into patient care decisions.

Furthermore, **we recommend a review of vendor liability laws to align responsibility with control.** If a provider utilizes a sophisticated AI product to guide care—a product that has been touted as effective and safe when used appropriately—the liability for any harm or bias caused by the tool itself should fall also to the vendor who developed and tested the product, not solely the clinician who deployed it. This shared accountability is necessary as vendors currently possess the exclusive knowledge of an AI tool’s underlying architecture, training data, and potential failure points. **After deployment, vendors must be required to provide immediate and transparent disclosure of any uncovered bias, potential harm, or negative findings related to their product as soon as it arises.**

Finally, **we strongly recommend that regulators create data standards for vendors to ensure that AI systems are built on interoperable, high-quality information that will lead to reliable and ethical algorithms.** As the industry moves farther in the direction of interoperability, ASTP/ONC should ensure that AI systems will not stymy or slow this advancement.

Include Health Plans in Regulation

The RFI appears to overlook a critical and increasingly prevalent application of AI in the healthcare ecosystem: its utilization by health insurance plans and payers. Specifically, there is no mention or substantive exploration of how health plans are currently deploying AI tools, particularly predictive models and algorithms, to inform or automate process decisions regarding the approval or denial of clinical care.

This is a significant omission, as the use of AI in utilization management is already widely in practice. Payers already are adopting these systems to rapidly process prior authorization requests, review claims, and predict the necessity or appropriateness of specific medical services, procedures, or prescriptions. While the intent may be to streamline administrative burdens and ensure cost-effective care, the deployment of these “black box” systems raises profound ethical, legal, and patient safety concerns. In addition, these decisions directly impact care delivery and clinical decisionmaking by choosing which products and procedures are covered by a patient’s health plan and reimbursable to providers. **We recommend that**

ASTP/ONC work with the appropriate regulatory agencies to ensure that health plans and payers are held to a high standard of quality, transparency, and appropriateness when using AI tools in utilization management or other decisions that impact patient care.

We appreciate ASTP/ONC's consideration of our comments. If you have any questions, please contact me at Rachel.Tanner@CommonSpirit.org. We look forward to working with you as part of a future technical panel and in future rulemaking.

Thank you,

/s/

Rachel Tanner, M.Jur.
System Vice President, Regulatory Affairs

CommonSpirit™

CommonSpirit Health is one of the largest nonprofit health systems with 150,000 employees and over 25,000 physicians and advanced practice clinicians and 45,000 nurses serving individuals and communities across 25 states in more than 137 hospitals and 2,300 care sites, including myriad long-term care facilities, home health organizations, clinics, and community service organizations. As a faith-based system, CommonSpirit seeks to create healthy communities with a special focus on those individuals who are medically and/or socially vulnerable.